

HEALTH PROFESSIONS APPEAL AND REVIEW BOARD

PRESENT:

Taivi Lobu, Vice-Chair, Presiding
Sheldon Cohen, Board Member
James Dault, Board Member

Review held on January 6, 2016 at Timmins, Ontario

IN THE MATTER OF A COMPLAINT REVIEW UNDER SECTION 29(1) of the *Health Professions Procedural Code*, Schedule 2 to the *Regulated Health Professions Act, 1991*, Statutes of Ontario, 1991, c.18, as amended

B E T W E E N:

A.A.

Applicant

and

F.T., R.Ph

Respondent

Appearances:

The Applicant:	A.A.
The Respondent:	F.T. R.Ph (by teleconference)
For the Respondent:	Craig R. Colraine and Matthew Di Giovanni, Counsel
For the Ontario College of Pharmacists:	Tracy Wills (by teleconference)

DECISION AND REASONS

I. DECISION

1. It is the decision of the Health Professions Appeal and Review Board to confirm the decision of the Inquiries, Complaints and Reports Committee of the Ontario College of Pharmacists to take no further action.

2. This decision arises from a request made to the Health Professions Appeal and Review Board (the Board) by A.A. (the Applicant) to review a decision of the Inquiries, Complaints and Reports Committee (the Committee) of the Ontario College of Pharmacists (the College). The decision concerned a complaint regarding the conduct and actions of F.T., R.Ph (the Respondent). The Committee investigated the complaint and decided to take no further action.

II. BACKGROUND

3. The Applicant has been a customer at the Loblaw Pharmacy (the pharmacy) in Timmins, Ontario since at least 2009. The Respondent has been the designated manager of the pharmacy. The Applicant has had a number of concerns relating to the confidentiality of his health information with the pharmacy.
4. The Applicant cites a specific incident that occurred in 2010 which he considers indicative of his ongoing interactions with staff at the pharmacy. When the Applicant attended at the pharmacy to pick up a prescription, the Respondent stated that she needed to telephone the prescribing physician to clarify the script because it appeared to have been changed since the previous dispensing. The Applicant states that the Respondent spoke loudly so that other pharmacy staff and customers overheard the conversation.
5. The Respondent also has had difficulties with the manner in which prescriptions have been delivered to his home - noting that prescription information has been attached to the outside of the bag and visible to anyone who chose to read it.
6. In addition, pharmacy staff have required the Respondent to sign for the receipt of some of his prescription drugs, specifically OxyContin in a public place where customers could witness his signing.

The Complaint and the Response

7. The Applicant complained:

- a) When he was at the pick-up counter in the Pharmacy, the Respondent “blurted” the name and specifics of his medication loudly enough for shoppers who were 20 feet away to hear. He stated that all Pharmacy staff speak openly too loudly;
- b) Pharmacy staff stapled a dispensing and delivery label, his official prescription receipt, and a list of his last fills and fills left, outside the paper bag containing his medication at delivery. He related that the documents outside the bag divulged his physician’s name, medication, fills left, and address. He asserted that only the physician, patient, and pharmacist should have access to the official prescription receipt and the document exhibiting a patient’s last fills and fills left, but alleged that anyone could have seen his private information. He claimed that having this information attached to the bag constituted a breach of privacy, and stated that it is illegal to divulge the information in question;
- c) Patients must sign for their medication in full view of other customers. In a further communication, he related that being required to sign a form in public every time he picks up narcotics announces the fact the he is picking up a narcotic, and makes him a target for robbery;
- d) Pharmacists and pharmacy assistants are “informants that breach privacy legislation; and
- e) The “same loose practices are rampant at all seven pharmacies” that he visited.

8. The Respondent submitted information in response to the complaint including the following:

- She did not recall the 2010 interaction and therefore explained how she provides counselling when there is a change of dosage or instructions.
- Counselling occurs in an area separated from the cash counter between two wickets meant to increase privacy, or on the other side of the cash counter five feet away where there is a separating wall. These areas are considered semi-private.

- Pharmacy staff regulate their voices and maintain close proximity to the client in order not to be overheard. However, if a patient is having trouble hearing the Respondent will raise her voice in order to be heard.
- In 2010, customer seating was 15 feet away from the counselling areas but are closer now because of a new freezer. Nevertheless, the distance still ensures privacy.
- Pharmacy staff will ask other customers to stand back if they feel they are too close to the counselling area or will move a patient to another area if necessary.
- As per her training and the need for clarity the Respondent always states the name of the medication and dosage when there has been a change. She indicated that she called the Applicant's physician because she found the instructions on the label to be unclear.
- The Ontario Narcotic Monitoring System (ONMS) requires patients to show identification when picking up narcotic, controlled and targeted drugs. Pharmacy staff determined that a signature is also required to prevent fraudulent behavior. Recently however, their system has been updated and they no longer require signatures.
- Pharmacy staff are not in a position to set up private space for signatures or to gather identification unless the patient asks. If requested, staff can bring the patient into the pharmacy's counselling room but this could not be done for every patient because the pharmacy does not have the manpower.
- Pharmacy staff and head office are working on improvements to privacy.
- Regarding the Applicant's confidential health information being stapled to the outside of the bag, the Respondent explained that someone would have to lift up a piece of paper in order to view the name of the drug on the page beneath the top page. The Respondent submitted that if a patient is dissatisfied with a procedure at the pharmacy they can speak with her about it and she would try to accommodate their request. As of May 2014, the name of the medication is no longer on the outside of the bag.
- When a prescription is delivered, the delivery driver does not have access to the name of the medication being delivered as this information is not on the outside of

the bag. The delivery service has been with the pharmacy for over 20 years and is very professional.

The Committee's Decision

9. The Committee investigated the complaint and decided to take no further action.

III. REQUEST FOR REVIEW

10. In a letter dated January 9, 2015, the Applicant requested that the Board review the Committee's decision.

IV. POWERS OF THE BOARD

11. After conducting a review of a decision of the Committee, the Board may do one or more of the following:
 - a) confirm all or part of the Committee's decision;
 - b) make recommendations to the Committee;
 - c) require the Committee to exercise any of its powers other than to request a Registrar's investigation.
12. The Board cannot recommend or require the Committee to do things outside its jurisdiction, such as make a finding of misconduct or incompetence against the member, or require the referral of allegations to the Discipline Committee that would not, if proved, constitute either professional misconduct or incompetence.

V. ANALYSIS AND REASONS

13. Pursuant to section 33(1) of the *Health Professions Procedural Code* (the *Code*), being Schedule 2 to the *Regulated Health Professions Act, 1991*, the mandate of the Board in a

complaint review is to consider either the adequacy of the Committee's investigation, the reasonableness of its decision, or both.

14. The Board has considered the submissions of the parties, examined the Record of Investigation (the Record), and reviewed the Committee's decision.

Adequacy of the Investigation

15. An adequate investigation does not need to be exhaustive. Rather, the Committee must seek to obtain the essential information relevant to making an informed decision regarding the issues raised in the complaint.
16. The Committee obtained the following documents:
 - the Applicant's letter of complaint and subsequent communication from the Applicant including diagrams and photos illustrating the pharmacy layout and various detailed suggestions on how to improve privacy in pharmacies
 - the letter of response and subsequent communications with the Respondent;
 - the Applicant's response to the Respondent's response; and
 - the pharmacy's prescriptions records and Medical History Report for the Applicant.
17. At the Review, the Applicant submitted that the Committee's investigation was inadequate since it did not physically investigate whether the semi-private areas were capable of containing sound thereby maintaining confidentiality. Nor did it investigate the voice levels or the content of staff interactions with patients at the pharmacy and at pharmacies in general. The Applicant wanted to know if the Committee had checked to see if the pharmacy had a private room and how it was offered to patients.
18. Counsel for the Respondent submitted that since the Applicant did not report his complaint at the time to the Respondent or anyone else, there was no possibility of any

records, such as an incident report being generated pertaining to the events in question. Since the Respondent could not reasonably be expected to remember the events in 2010, complained about by the Applicant for the first time in April 2014, the Committee could only rely upon the Respondent's description of her and her staffs' normal counselling practice and the policies in place at the Loblaw's pharmacy.

19. The Board notes that much of the Applicant's complaint addressed the overall patient confidentiality in Ontario pharmacies. The Committee, being under the *Code*, is mandated to address the Applicant's complaint as it relates to the Respondent's conduct as the designated manager of the Loblaw's pharmacy in relation to the events in issue. The Board notes that the information available to the Committee in considering this matter was the Respondent's detailed description of her and her staffs' counselling practices and the policies and Practice Standards that the Committee consulted. If the Applicant had complained at the time of the incident, pharmacy staff would have been required to file an incident report but this did not occur.
20. The Board finds the Committee's investigation obtained the essential information regarding the issues raised in the complaint. There is no indication of further information that might reasonably be expected to have affected the decision, should the Committee have acquired it. Accordingly, the Board finds that the Committee's investigation was adequate.

Reasonableness of the Decision

21. In considering the reasonableness of the Committee's decision, the question for the Board is not whether it would arrive at the same decision as the Committee, but whether the Committee's decision can reasonably be supported by the information before it and can withstand a somewhat probing examination. In doing so, the Board considers whether the decision falls within a range of possible, acceptable outcomes that are defensible in respect of the facts and the law.

(a) The Respondent “blurted” the details of the Applicant’s prescription loud enough for other patients to hear.

22. The Applicant recounted that in 2010 while at the counter to pick up his medication, the Respondent, “speaking loud enough for people to hear 20 feet away” and using his name, explained that his prescription for OxyContin had been reduced and that she had spoken with his physician, all of which breached the confidentiality of his personal health information.
23. The Respondent explained that her practice is to counsel patients in the semi-private counter areas where the transmission of sound is limited by privacy panels. She only speaks as loudly as required for the patient to hear. As per her training, she states the name of the medication and details of the prescription particularly when there has been changes in the medication, as occurred on that date, in order to provide clarity and reduce risk.
24. The National Association of Pharmacy Regulatory Authorities (NAPRA), in the *Model Standards of Practice for Canadian Pharmacists* requires that pharmacists ensure that the confidentiality of patient information be maintained. The *Supplemental Standards of Practice for Schedule II and III Drugs*, Standard #2, page 3, states that a “pharmacist shall consult in an area where the patient feels comfortable. This could be a private consultation area, a semi private area, or a quiet, secluded portion of the pharmacy”. A patient can request the level of privacy they feel comfortable with from the above range. The Applicant did not request a greater level of privacy.
25. The Committee decided to take no further action on this aspect of the complaint.
26. The Board notes that Respondent would not be expected to recall an incident which was not made subject of a complaint until years after the fact. There was no apparent indication at the time for creating any incident report about the event in question. In such

circumstances, the Respondent could not be expected to provide a meaningful response to the concern other than to reference her standard practice. As the information provided by the Respondent about her practice was in accordance with College expectations, the Board finds reasonable the Committee's decision to take no further action in relation to this issue of the complaint.

(b) The Applicant's confidential personal health information was stapled to the outside of his medication bag

27. The Applicant submitted that attached to the outside of his medication bag, as one of three individual slips of paper was the name of his physician, medication, fills left and his address which constituted an illegal breach of privacy. He maintained that this could pose a safety risk by identifying him as a target for robbery in light of the fact that his medication is often OxyContin.
28. The Respondent submitted that the slip of paper containing a patient's confidential information is always underneath one of the other two generic slips in order to maintain confidentiality. The only way to view the information would be to physically lift the other slip and look underneath. When a prescription is delivered, the confidential slip is inside the bag. The Respondent advised that a new computer system has allowed the pharmacy to only print the patient's name, address, prescription number and balance owing on the outside of the bag. In addition, the Respondent stated that if a patient was dissatisfied with any process such as this, he or she could raise this with her and she would address it to the patient's satisfaction, as she has done in the past.
29. The Committee reviewed confidentiality measures at the pharmacy, observing that all pharmacy employees sign confidentiality agreements so that they are able to share information. It commented that there was value in such an arrangement, as multiple staff members are required to work on a single prescription and such staff communication allowed for checks and double checks, which improve patient safety. The Committee did note that although the Applicant alleged he received a prescription delivery which had

confidential information stapled to the outside of the bag, the evidence he submitted in this respect was from a Walmart pharmacy and not the Respondent's pharmacy.

30. The Board finds the Committee's decision to take no further action on this aspect of the complaint to be within a range of reasonable outcomes and supported by the information in the Record.

(c) Patients must sign for narcotic medication in full view of other customers

31. The Applicant complained that having to sign for his medication each time he picked it up also identified him as a narcotic user targeting him for theft and possibly stigmatizing him in the eyes of some others. He argued that although he understood the need to identify himself, he was not technically required to sign, but only to show identification.
32. The Respondent explained the requirements imposed by the Ontario Narcotic Monitoring System (ONMS) which require that pharmacists collect identification from patients when they pick up narcotics. The Respondent stated that a patient's signature was a safeguard the pharmacy put into place to verify the identification, but added that as a result of the new computer system they could now comply with ONMS requirements without obtaining a signature. Counsel for the Respondent further submitted that this requirement is also contained in section 153 of the *Drug and Pharmacies Regulation Act*. R.S.O. 1990, c.H.4 (DPRA) which requires that "the designated manager of every pharmacy shall keep or cause to be kept a record of every purchase and sale of a drug referred to in the Schedules to the *Controlled Drugs and Substances Act* (Canada) or the Schedule to the *Narcotic Control Regulations* (Canada)..."
33. The Committee took no further action on this element of the complaint explaining that "there is a requirement and a public policy reason to require a signature when picking up narcotics ... it is important to ensure that narcotics are not diverted and that they are dispensed to the person to whom they are prescribed."

34. The Board finds that the Committee's analysis and disposition of this aspect of the complaint is reasonable.

(d) and (e) Pharmacists and pharmacy assistants are “informants that breach privacy legislation and the “same loose practices are rampant at all seven pharmacies” that he visited.

35. Included in the Applicant submissions under this heading are allegations such as “all pharmacy staff speak to loudly” and “all pharmacy staff staple confidential slip to the outside of medication bags.”
36. The Applicant would like a review of systemic safeguards and practices to assure patient confidentiality in pharmacy settings. However the complaint process under the *Code*, is intended to address the practice of individual members. In this case, the Committee was mandated to address the Respondent's practice. As has been outlined, the Board finds that the Committee has reasonably done so.

VI. DECISION

37. Pursuant to section 35(1) of the *Code*, the Board confirms the Committee's decision to take no further action.

ISSUED May 5, 2016

Taivi Lobu
Taivi Lobu

Sheldon Cohen
Sheldon Cohen

James Dault
James Dault